



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : سلوى محمد محمود عبد المطلب

المحافظة : القاهرة الحي / المدينة : الزاوية الحمراء العنوان : //

رقم البطاقة : 28103120101101 رقم التليفون : 01151636285 تاريخ تسجيل الإستمارة : 21-09-2026

العمر (تاريخ الميلاد) : 12-03-1981 المهنة : ربه منزل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي العلاج بعقار (Capecitabine 500mg) قسم العلاج : الاورام [14149]

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... /د. /د. /د. /د. /د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: **سلوى محمد محمود عبد المطلبى**
National ID: **28103120101101**



تقرير طبي

المسجد

تحية طيبة وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الطول :
-- -- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد انه

المریضة تعاني من ورم غير حميد بالبدن و تحتاج الي Capecitabine therapy

الوقت :

-- --

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين

IK3739792



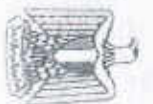
جمهورية مصر العربية
وزارة الداخلية
مصلحة الأحوال الشخصية

سليم
محمد محمود عبدالمطلب
مسكن الزاوية الجديدة - ب ١٨٦ م ٥
الزاوية الحمراء - القاهرة

٢٨١٠٣١٢٠١٠١١٠١



٢٠٣٠/٠٦ - ٢٨١٠٣١٢٠١٠١١٠١
حصة على بسطن نائب المحافظ
التي منطقة
محمد علي محمد علي
البلدية سارية حتى ٢٠٣٧/٠٦/٠٧



Patient's Name: Salwa Mohammed Mahmoud
Date: 2026-05-21
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

**MULTISCLICE NON CONTRAST C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS REVEALED:**

I- CHEST:

- Left mastectomy status
- Bilateral lower lobar fibrotic strand
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No dilated biliary radicles. Multiple bilobar hepatic focal lesions the largest is seen in the right lobe measuring 27 x 26 mm showed marginal enhancement
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Left ischial tuberosity sclerotic lesion
- D6 vertebra plana

Opinion:

Multiple hepatic focal lesions likely metastatic.

- ❖ Left ischial spine sclerotic lesion for bone scan.
- ❖ D6 vertebra plana
- ❖ Bilateral lower lobar fibrotic strands.

Dr. Asser Ahmed

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Dr.Assem Ahmed

MDT FORM

Patient Name	ملوى محمد محمود	Age	44
Sex	F	Special habits	
Family History		Comorbidity	MF PS2
Surgical History		Telephone	01151636285
Menopausal Status	Stopped since ...	Marital Status	Married
ID	28103120101101	Medical ID	3284/2025
Date	18/11/2025	Reason of MDT	Recurrent

Medical Report

Pathology: Recurrent Lt breast cancer IDC NST

T N M

S.M:

Metsworkup: CT CAP with contrast → Free
Bone scan → Free

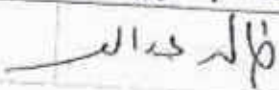
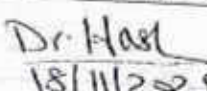
Doctors' notes:

2022 → Lt MRM (0/13, no residual tumor) → ER 7, PR 6, Her2 +ve, Ki67 20%,
received adjuvant hormonal & zoladex
Now → presented with BSM 4/2025 showing op. Bed 2 lesions & 2nd intercostal
space lesion related to lateral sternal border & osteochondral junction
5/2025 → tru cut biopsy show op. Bed lesion (IBC NST with ER 4/8, PR 2/8 <1%,
Her2 +3, Ki67 25%)
7/2025 → tru cut biopsy showing intercostal space lesion (MEet. epith. Likely
breast origin)

Recommendations :

MDT DECISION

Start herceptin + oral navelbine
Reasses after 3 - 6 months for possible resection of all
recurrent disease (oligo metastatic)

Name	د/ خالد عبدالعزيز	د/ هاشم عبدالرحمن	د/ ابو بكر حجازي
Signature		 18/11/2025	

Asked her to get BSM film for



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الاستشارة
سرطان
مصر

SR
Recd

MDT FORM

18/11/2025 G

Name	لبنى محمد	MRN	5284
Age	44 years	ID	25
Menopausal status	Stopped since	Phone	0151636285
NRS SCORE		Marital status	Married
BMI		Family history	Cancer
CO morbidity	MF. PS2	REASON OF MDT	Recurrent

Clinical Data

BX:

ER:

PR:

HER 2:

Ki67:

R

L

Sonmammography:

Recurrent Lt. B.C IDC NST

NACTH

Lt. MRM (0113 - No residual tumor)

Mets Workup:

Adj → Hormonal & Zoladex.

ER 7 Her 2 free
PR 6 Ki67 20%

Now presented with

BSM

4/2025

OP bed 2 lesions.

2nd intercostal space lesion

TNM

Recommendations:

related to left sternal border
(Costo chondral) infection

6/2025

→ Treat at bx

OP bed lesion

IDC (NST) G1

7/2025

→

→

→

Intercostal space lesion

ER 418 37

PR 26

Met epit. likely

Her 2 +3

<1%

Breast origin.

MDT OUTCOME & PLAN

(Yes)

Bone Scan → free

CT CAP e' Contrast → free

① Start Herceptin + oral
Navelbine

② Reassess after 3-6 months
for possible resect.

SIGNATURE all recurrent dx (oligo metastatic)

MEDICAL ONCOLOGY

SURGERY

RADIOLOGY

PATHOLOGY

Nutrition

Psychiatric

Dr. Hoss

18/11/2025

Signature

Patient Name: Salwa Mohamed Mahmoud Abd Al Mottaleb	Date: 10/12/2025
Exam: CT CAP+C (CT Chest Abd. Pelvis)	Age: 44 Year(s)
Medical ID: 3284/2025	Pat. ID: 133311

CT Chest with contrast

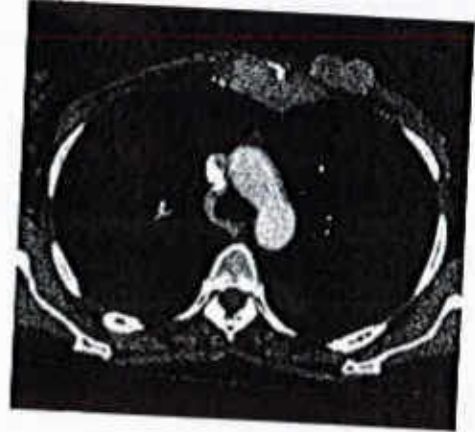
FINDINGS: status undergone left mastectomy ith operative two soft tissue mass infiltrating the pectoralis muscle measures about 21x14mm and about 21x20mm in diameters shows peripheral post contrast enhancement.

noted irregular soft tissue lesion infiltrating the left side of sternum and 2nd left costochondral junction with heterogenous post contrast enhancement, it measures about in diameters.

Another noted soft tissue lesion infiltrating the sternum and costochondral junction measures about 4.8x2.7cm in diameters

Lungs: few scattered atelectatic bands

- Trachea and large airways: Normal caliber and wall.
- Pleura: No pleural collections or lesions seen.
- Mediastinum and hila: few subcentemetric mediastinal lymph nodes.
- Heart and pericardium: Normal size and CT features.
- Vessels: Normal appearance of thoracic aorta and pulmonary vessels.
- Chest wall: D6 lytic lesion



bed
, they
the
4x4cm
3rd left

IMPRESSION:

- Known case of left breast cancer with operative bed and anterior chest wall soft tissue lesions likely metastatic deposits.
- D6 lytic lesion.

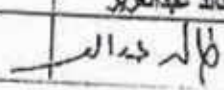
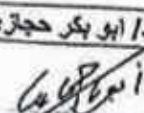
FINDINGS:

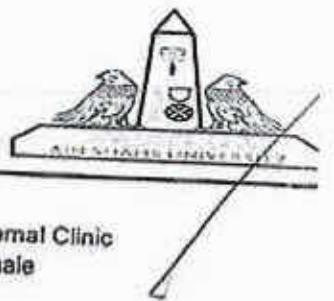
CT abdomen and pelvis with contrast

MDT FORM

Patient Name	سلوى محمد محمد	Age	44
Sex	F	Special habits	
Family History		Comorbidity	MF PS2
Surgical History		Telephone	01151636285
Menopausal Status	Stopped since ...	Marital Status	Married
ID	28103120101101	Medical ID	3284/2025
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Pathology:	Recurrent Lt breast cancer IDC NST
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Metsworkup:	CT CAP with contrast → Free Bone scan → Free
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MDT DECISION	Start herceptin + oral navelbine Reasses after 3 - 6 months for possible resection of all recurrent disease (oligo metastatic)

Name	د/ خالد عبدالعزيز	د/ هاشم عبدالرحمن	د/ ابو بكر حجازي
Signature		Dr. Hasm 18/11/2025	



Patient Name:	سنرى محمد محمود عبدالمطلب	Location:	External Clinic
MRN:	722456742	Sex:	Female
D.O.B:	12/03/1981		
IC No.:	28103120101101		

Surgical Pathology
Specimen

Specimen Id	2505061305	Collection Date	06/05/2025 01:36 PM
Accession No	SU-25-2914	Received Date	06/05/2025 01:36 PM
Sample Type	TISSUE		

Specimen

Tru-cut biopsy from left operative bed suspicious lesion.

GROSS DESCRIPTION

Multiple tan tissue cores, longest measured 0.8cm in length.TS.

MICROSCOPIC

Sections examined reveal fibrofatty tissue with adjoining muscle fibers infiltrated by a malignant neoplasm arranged in sheets, trabeculae and clusters. The cells display marked degree of nuclear pleomorphism and mitotic activity.

IMMUNOHISTOCHEMICAL RESULT

- ER: Moderate positive nuclear staining in 5% of tumor cells (Allred score 4/8).
- PR: Weak staining in less than 1% of tumor cells (Allred score 2/8).
- Her2neu: Strong complete membranous staining in more than 10% of tumor cells (Score 3+; Positive).
- Ki-67: Proliferation Index: 25%.

DIAGNOSIS

Left operative bed suspicious lesion in a known case of breast cancer with left sided mastectomy:

- Invasive breast carcinoma, NST (ductal), Grade III.

Responsible pathologist

Dr. Lobna Attia,
19/5/2025

Recurrent Breast Cancer

Reported and electronically endorsed by

Anatomical Pathology Unit

Validated on 20/05/2025 11:18 AM

Print Date: 28/05/2025 11:18 AM



Patient Name:	ملوى محمد محمود عبدالمطلب	Location:	External Clinic
MRN:	722456742	Sex:	Female
D.O.B:	12/03/1981		
IG No:	28103120101101		

Surgical Pathology
Specimen

Specimen ID	2507061130	Collection Date	06/07/2025 01:29 PM
Specimen No	SU-25-4468	Received Date	06/07/2025 01:29 PM
Sample Type	TISSUE		

Specimen
The site biopsy left 2nd intercostal space lesion.

GROSS DESCRIPTION
A single whitish tissue core, measured 1 cm in length.

MICROSCOPIC
Sections examined reveal fibrofatty tissue core infiltrated by malignant epithelial cells arranged in sheets, trabeculae and clusters, with no glandular differentiation. The cells display moderate degree of anaplasia. Surrounding stroma is desmoplastic. Scattered mitotic figures are noted.

DIAGNOSIS
Second intercostal space lesion, biopsy:
Metastatic epithelioid neoplasm likely of breast origin (in view of the patient's history).
For confirmation by GATA3 Immunohistochemical staining (not available in our lab nowadays).

Physicians
Dr. Ehsan Al Gammal
Dr. Yasmin Salah
06/07/2025
Reviewed by
Galia Elwan

Reported and electronically endorsed by
Anatomical Pathology Unit
Validated on 12/07/2025 12:25 PM

Print Date: 06/07/2025 11:58 AM



٢٥٦٧
١٤٤٤

40 Years - Female
2022-01-05 01:23 م
2022-01-19 10:20 ص

0-702	
ملف رقمي محمد محمود عبد المطلب	

PATHOLOGY REPORT

Specimen : Tru- cut biopsy from left breast lesion.

Gross :

Multiple tan tissue cores, ranging in length from 0.3 to 1.2cm , totally submitted in one block.

Microscopic :

serial sections examined reveal multiple adequately representative mammary tissue cores showing sheets and clusters of malignant looking ductal cells. The cells exhibit large, hyperchromatic nuclei with prominent nucleoli and evident mitotic activity . Stroma is desmoplastic.

Diagnosis :

- Tru- cut biopsy from left breast lesion:
- Invasive breast (duct) carcinoma , NST Grade II - III.
 - Core needle biopsy category : B5b.
 - Case for ER, PR, HER2neu, Ki67 IHC staining.

Angel Mohsen

Signature
Prof. Manal Fawzy.
Dr. Diana Bishara.



Handwritten signature/initials.

40 Years - Female
2022-01-18 01:23 م
2022-01-19 10:22 ص

0-702	
ملوى محمد محمود عبد المطلب	

PATHOLOGY REPORT

Specimen : Tru cut biopsy from left breast lesion for IHC.

Technique :

Immunostaining effectuated on 4 um thick paraffin sections from the paraffin block, processed in Benchmark GX (Ventana) and stained by primary antibodies against DAB as chromogen and hematoxylin as counter stain The sections were treated against ER, PR, Her2neu, Ki67.

Result :

ER: Positive immunostaining of neoplastic cells (Allred score 7).PR: Positive immunostaining of neoplastic cells (Allred score 6).Her2neu: Positive membranous immunostaining of neoplastic cells(3+).Ki67: Positive immunostaining in 20% of neoplastic cells.

Angel Mohsen

Signature
Prof. Ola Nada.



Age / Gender 41 Years - Female

Visit No. 0-1292

Registered Date 2022-10-09 11:53 م

Patient Name سلوى محمد محمد عبد المطلب

Reporting Date 2022-10-18 12:22 م

PATHOLOGY REPORT

Specimen : Left MRM.

Gross :

Left MRM measured 17x11x2cm with skin ellipse measured 15x11cm. Serialling revealed three inserted clips, The 1st was located at the 7th serial inferiorly surrounded by rubbery greyish white area measured 3x1.5x1cm, located 3cm from deep margin.

The 2nd was located at the 8th serial, retro- areolar, surrounded by a rubbery area measured 1.5x1.5x1cm, located 4cm away from deep margin.

The 3rd was located at 9th serial superiorly (12 o'clock) showing a rubbery greyish white focus measured 1x1x1cm located 2cm from deep margin and 1cm from superior margin, surrounded by a rubbery greyish white area occupying from the 8th to 14th serials measured 6x6x5cm.

Very careful dissection of the minimally represented axilla revealed 13 nodules longest was 2cm in longest dimension.

1) Representative from serial 7 (at clip).

3) Area in between serial 7&8.

5) Deep margin of serial 8.

7) Representative from serial 9 (at clip).

9) Deep margin of serial 9.

13) Nipple & areola.

Fe gross:

17,18,19) Representatives from breast.

2) Deep margin of serial 7.

4) Representative from serial 8 (at clip).

6) Area in between serials 8&9.

8) Representative from serial 9 (at clip) + superior margin (inked).

10,11,12) Rubbery area surrounding serial 9.

14,15,16) Lymph nodes.

Diagnosis :

Microscopic& diagnosis:

Left MRM In a known case of breast carcinoma who received neoadjuvant chemotherapy:

- No microscopic residual invasive carcinoma in the examined sections for further clinico radiological correlation.

- DCIS: Not identified in the examined sections.

- Deep margin: Free of malignancy.

- Nipple and areola: Free of malignancy.

- Treatment effect in the breast: Areas of fibrosis, vascular proliferation, lymphocytes, histiocytes and giant cell reaction along with atypia in ductal cells are present.

- Left axillary lymph nodes:

* Number of lymph nodes dissected: 13.

* Number of lymph nodes with metastasis: Zero/13.

- Treatment effect in the lymph nodes: Vascular proliferation and hemosiderin laden macrophages are seen.

- Additional pathological findings: Fibrocystic changes, Duct ectasia with periductal inflammation and usual ductal hyperplasia are seen.

- Pathologic stage : ypT0 N0 Mx for clinico radiological correlation.

Sara Saad

Signature

Prof. Lobna Abd EL Fatah.

17/10/2022



Patient Name	Salwa Mohammad Mouteleb-Mahmoud Abd El	PID	722456742	D/T	17-04-2025
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SONOMAMMOGRAPHY OF THE RIGHT BREAST

Procedure: Digital mammography and real time ultrasonography of the right breast was performed+ ultrasound of the left surgical bed.

Findings:

- Status post left sided mastectomy:
- The left surgical bed shows two adjacent well defined soft tissue lesions with irregular borders, measuring 1.3 x 0.7 cm and 1 x 0.5 cm, with detected internal arterial vascularity.
- The left 2nd intercostal space shows a well defined lobulated suspicious looking soft tissue lesion, it is seen directly related to the lateral sternal border and the costo-chondral junction, measuring 2 x 2 cm, indenting the pleural line, with detected arterial vascularity.
- The right breast displays normal skin thickness with normal appearance of the nipple.
- The breast parenchyma is predommixed fibro-fatty (ACR breast density B).
- No suspicious solid or cystic masses or clusters of microcalcifications were detected in the right breast.
- No suspicious axillary lymphadenopathy.

Impression:

- LEFT SURGICAL BED TWO ADJACENT SOFT TISSUE LEISONS AS DESCRIBED, FOR HISTOPATHOLOGICAL VERIFICATION.
- LEFT 2ND INTERCOSTAL SPACE SUSPICIOUS LOOKING SOFT TISSUE LESION, FOR HISTOPATHOLOGICAL VERIFICATION.

Much obliged,
Basma Saady, MSc

↳ BIRADS 4 B
(Not for surgical bed)
BIRAS ??

Email: feedbackradiology@gmail.com

مستشفيات جامعة عين شمس - شارع رمسيس - الحاسية القاهرة

NAME: SALWA MOHAMED MAHMOUD
DEAR SIR

PATIENT ID: 1331753
STUDY DATE: 05/07/2025

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

History

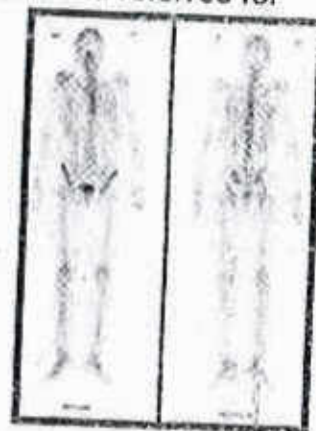
- 44-year-old female patient presented with history of breast cancer and referred for metastatic workup.

Technique

- Anterior and posterior whole-body scans were acquired 2 hours post injection of 20 mCi of the tracer.

Findings

- No evidence of bone secondaries could be seen.
- Degenerative changes are noticed at different levels of the spine and around big joints.



IMPRESSION

- The skeleton is free from metastases.

MUCH OBLIGED

PROF DR: MAGED ABDEL GALIL

Name: Mrs. Salwa Mohamed Mahmoud Abdel-Motelb

Date: 18/8/2026

ID: 2226028524

Acc.: 5289744

Referring Prof. Dr: Hospital El-Sheikh Zayed El-Nhian

MRI of the dorsal and lumbar spine with IV contrast using different

pulse sequences in different planes, revealed:-

Clinical history: known case of managed metastatic breast cancer:

L-Dorsal finding:

- Structural collapse with vertebra plana of the D6 vertebral body and dumbbell shaped intraspinal and pre-vertebral soft tissue component. Measures respectively 3.2x1 cm and 3.7 x0.6cm
- It markedly compress the spinal cord at this level with consequent intramedullary patchy area of high T2 signal.
- Consequent exaggerated dorsal kypnosis.
- Multiple scattered infiltrative variable sized low T1/T2 and high STIR signal intensities lesions, seen involving D10, D11 vertebral bodies they show intense heterogeneous post contrast enhancement.
- Normal height and signal intensity of the dorsal discs.
- No posterior disc lesions.

II-L-SS Findings:

- Multiple scattered infiltrative variable sized low T1/T2 and high STIR signal intensities lesions, seen involving L1 (left pedicle and transverse process with encroachment upon the spinal canal and left neural exit foramen), L2, L3, S1, S2 and S3 level, they shows intense heterogeneous post contrast enhancement.

Name:

Mrs. Salwa Mohamed Mahmoud Abdel-Motelb

- Associated left sided sacral segments (S2 and S3) extra-osseous soft tissue component which measures about 3.5x4.7 cm in maximum axial encroaching upon the left exiting nerve roots and sacro-iliac joints.
- Small intraspinal soft tissue component of S1 vertebral body posterior cortex.
- Reduced T2 signal intensity of L4/5 disc.
- No significant posterior disc lesions.
- Normal conus and cauda.

Conclusion:

Case of malignant breast cancer shows:

- Multilevel dorsal and lumbar vertebral metastatic deposits with extra-osseous soft tissue component, intraspinal canal extension and cord myelopathy as well as L1 and S2/S3 neural compromise as described.

Best regards;

Dr. Mohammed Mohsen, MD

Typed: M.sh 21/08/26 12:18:49 PM

Dr.
Elia Anis Ishak

M. Sc. Ph. D. Pathology - Prof. of Pathology , Cairo University
Fellow, Armed Forces Institute of Pathology , USA
Member, United States Canadian Academy of Pathology

Dr.
Shady Elia Anis

M. Sc. MD Pathology , Ass. Prof. of Pathology , Cairo University
Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : **سليمة محمد محمود عبد الحظي**

Analysis ID :  1088325

Age : 45 Y : 5 M : 1 D Gender : Female

Conf. Code : 2786

Referred By Prof. Dr. : **محمد مصطفى**

Clinical Diagnosis : Patient with history of breast cancer underwent radical mastectomy presented with D6 lesion.

Nature of Specimen : Biopsy

Patient ID : 793836

Receiving Date : 2026-08-22

PATHOLOGY REPORT

Gross :

Two containers were received as:

- 1) Multiple soft grayish tan tissue pieces together with hard bony tissue pieces collectively measured 1.5x1.5 cm, totally submitted.
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Microscopic :

Sections examined from the Both specimens received revealed pieces of tumor tissue formed of groups of malignant epithelial cells showing moderate nuclear pleomorphism and hyperchromasia, arranged in nested pattern and solid sheets.
There is moderate desmoplasia.
Portion of bone is infiltrated.

Diagnosis :

D6 lesion, Biopsy , METASTATIC CARCINOMA.

- PLEASE CORRELATE WITH HISTORY.

- CASE FOR IMMUNOPHENOTYPING

Prof. Dr. Shady Elia Anis

Prof. Dr. Elia Anis Ishak



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تقرير طبي t

اسم المريض /	سلوي محمد محمود عبدالمطلب		
الرقم الطبى :	١٩٠٠٨-٢٦ / ١٥١٣٣٤٣٥٩		
تاريخ الكشف :	٤٥ سنة		
القسم :	جراحة العمود الفقري		
رقم إذن القبول	٧-ب		
المعاملة المالية :	طوارئ ٤٨ ساعة		
	٢٠٢٧٨٣٩٤		

الهيئة

تاريخ الدخول: / / ٢٠٢ تاريخ الخروج: ٨/٢٦ / ٢٠٢ الوقت:
 موجه الى السيد /

يتوقع الكشف الطبى على السيد / سلوى محمد محمد
وجد أنه يعانى من : ورم بالعمق است العنبر، مضاء على الصلابة
لحمية القدرة على التحكم بالبول أو البراز هائبة الانقباض وتتم
الفتاة العنبرية العنبرية وتشتت العنبرية في ذلك
وقد أجرى له : الفارصة

4. $\frac{1}{2} \leq \frac{1}{2} \leq \frac{1}{2}$ (continued)

(ب) انجمن اعلیٰ تعلیم:

العلاج اللازم :

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- آخری :

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	الجنس : F	تاريخ الدخول : 439	
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تقرير فحص عينة باثولوجي

Microscopic picture

ملوك
 Patient 43 yrs female. Known case of Breast cancer who undergo Radical mastectomy. Patient MRI show lesion D 6. Undergo decompression + Fixation T4 → T8. Biopsy is taken from the tumor for pathological examination.

في

DIAGNOSIS

الوقت / التاريخ : ٢٠٢ / /

التوقيع :

Name: Mrs. Salwa Mohamed Mahmoud Abdel-Motelb

Date: 18/8/2026

ID: 2226028524

Acc.: 5289744

Referring Prof. Dr: Hospital El-Sheikh Zayed El-Nhian

MRI of the dorsal and lumbar spine with IV contrast using different

pulse sequences in different planes, revealed:-

Clinical history: known case of managed metastatic breast cancer:

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- Multiple scattered infiltrative variable sized low T1/T2 and high STIR signal intensities lesions, seen involving D10, D11 vertebral bodies they show intense heterogeneous post contrast enhancement.
- Normal height and signal intensity of the dorsal discs.
- No posterior disc lesions.

II-L-SS Findings:

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تاريخ الكشف :	١٩٠٠٨-٢٦ / ١٥١٣٣٤٣٥٩	القسم :
طوارئ	جراحة العمود الفقري	رقم إذن القبول :
٤٨ ساعة	٢٠٢٧٨٣٩٤	المعاملة المالية :

الهيئة

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تقرير فحص عينة باثولوجي

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Patient Name : شادي محمد محمود عبد الحظي

Analysis ID : 1088325

Age : 45 Y : 5 M : 1 D Gender : Female

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Patient ID : 793836

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وقد أجرى له : الجراحة

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الوقت / التاريخ : ٢٠٢ / /

التوقيع :

Name : Salwa Mohammed Mahmoud

Date : 12-09-2026 03:00

Branch : Dokki

Medical No. : 459956

Ref.Physician Prof : Goustav Roussey Egypt

Ins.Company : تعافد مستشفيات

Tc-99m MDP SKELETAL SCINTIGRAPHY

History

45 year-old female patient, with history of treated left breast cancer, previous history of plate and screw for DV6 and referred for skeletal survey.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post injection of 20 mCi of the tracer.

Findings

- Heterogenous radiotracer distribution is noticed opposite DV4 down to DV8, likely attributed to previous surgical intervention for fixation of DV6.
- Uneven active tracer uptake is noticed opposite the proximal part of the right femur, for CT correlation if clinically warranted.
- Otherwise uniform radiotracer uptake is noted all over the surveyed skeleton with no evidence of tracer uptake abnormalities that would account for bone secondaries.

IMPRESSION

- No definite osseous metastasis could be depicted on scintigraphic basis.

Much obliged

Dr. Alaa Nagy, MD,

Nuclear medicine

Patient's Name: Salwa Mohammed Mahmoud
Date: 2026-09-10
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

**MULTISLICE POST CONTRAST C.T. EXAMINATION OF THE NECK,
CHEST, ABDOMEN AND PELVIS AFTER ORAL AND I.V. CONTRAST
INJECTION REVEALED:**

Compared to the previous CT scan performed on 21-5-2026

I- NECK:

- Normal appearance of the naso-, oro- and hypopharynx with preserved parapharyngeal fat planes.
- Intact laryngeal skeleton with no glottic, supra or infra glottic masses.
- Normal course and calibre of the carotid and jugular vessels.
- Normal CT appearance of the submandibular and parotid salivary glands showing homogenous enhancement. No focal solid or cystic lesions.
- Few subcentimetric bilateral upper deep cervical and submandibular LNs are noted; likely non-specific.
- No pathologically enlarged cervical lymph nodes.

II- CHEST:

- Left mastectomy status
- No appreciable changes regarding the two soft tissue masses infiltrating the pectoralis muscle measure 10x11mm & 11x11mm
- Still noted soft tissue lesion infiltrating the sternum and costochondral junction measures 21x22mm
- No pulmonary nodules, masses or cavity lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

I- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No dilated biliary radicles.

Patient's Name: Salwa Mohammed Mahmoud
Date: 2026-09-10
Referred by: Prof. Dr. no doctor
Hospital: Goustave Roussey International Egypt

- Progressive course regarding the size and number of the previously noted multiple bilobar hepatic focal lesions the largest is seen in the right lobe segment VII measuring 63 x 52 mm showed marginal enhancement
- Both kidneys are normal size, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulac or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Still seen left ischial tuberosity sclerotic lesion
- D6 vertebra plana (progression of the lytic lesion)
- Newly develop multiple dorsal, lumbar and sacral lytic lesion with the largest seen in the left aspect of the sacrum measuring 49 x 48 mm

Opinion:

❖ Overall progressive course since the last study

Dr. Assem Ahmed